2024 PSYC489 Research Project

Psychedelic Effects Under Serotonergic Antidepressants: A Qualitative Analysis of User Reports

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Abstract

The therapeutic potential of classical psychedelics in treating Major Depressive Disorder (MDD) has garnered increased clinical attention, yet their interaction with serotonergic antidepressants remains inadequately characterised. This study examined the phenomenological and safety implications of combining lysergic acid diethylamide (LSD) or psilocybin with serotonergic antidepressants through thematic analysis of online naturalistic experience reports (n=41). Analysis revealed seven major themes encompassing acute effects (altered intensity, individual response variability), management strategies (safety behaviours, dose adaptations), qualitative dimensions (experiential changes, meaning-making), and adverse events. While most participants (63.4%, n=26) experienced attenuated psychedelic effects, a notable subset reported unaltered (14.6%, n=6) or enhanced (12.2%, n=5) effects. Many participants (22.0%, n=9) described profound mystical-type experiences characterised by unity, transcendence of space/time, and noetic quality, despite commonly reporting attenuated sensory effects; these occurred more frequently with psilocybin (23.1%, n=6 of 26 psilocybin reports) than LSD (14.3%, n=3 of 21 LSD reports). Clear indicators of serotonin toxicity occurred in 9.5% (n=2) of LSD reports compared to no distinct cases with psilocybin, though one possible case (3.8%) was reported. Similarly, severe psychological crises requiring emergency intervention were more common with LSD (9.5%, n=2 of 21 LSD reports) than psilocybin (3.8%, n=1 of 26 psilocybin reports). Participants reported difficulty accessing reliable drug interaction information. As a result, some engaged in potentially unsafe strategies such as extreme dose escalation (22.0%, n=9), abrupt antidepressant discontinuation (17.1%, n=7), and reliance on peer-based risk management approaches. These findings suggest that psilocybin may have a more favourable tolerability profile than LSD when combined with serotonergic antidepressants and indicate that SSRI discontinuation protocols in psychedelic-assisted therapy warrant reconsideration.

Major Depressive Disorder and Current Treatment Approaches

Major Depressive Disorder (MDD) represents a significant global health burden, affecting millions of individuals and contributing substantially to decreased quality of life (Ferrari et al., 2022). While conventional treatments such as psychotherapy and pharmacological interventions show efficacy, a considerable proportion of patients continue to experience treatment-resistant symptoms or severe medication side effects (Carhart-Harris & Goodwin, 2017). Selective serotonin reuptake inhibitors (SSRIs) and serotonin-norepinephrine reuptake inhibitors (SNRIs) are currently the most widely prescribed classes of antidepressant medications (Luo et al., 2020). SSRIs act by blocking the reuptake of serotonin by the serotonin transporter (SERT), while SNRIs block both serotonin and norepinephrine uptake, increasing synaptic availability of these neurotransmitters. Sustained elevations in serotonergic activity are thought to alleviate depressive symptoms through 5-hydroxytryptamine (5-HT) system remodelling and subsequent altered gene expression (Vahid-Ansari & Albert, 2021). Common serotonergic antidepressants include fluoxetine, sertraline, and escitalopram.

Psychedelics as Novel Therapeutic Agents

The potential application of psychedelic agents in treating mental health conditions has gathered increased attention in recent years, reigniting interest in a field largely dormant since the 1970s. Classical psychedelics – including lysergic acid diethylamide (LSD) and psilocin (the active metabolite of psilocybin) – are a class of psychoactive substances that act as direct partial agonists at 5-HT2A receptors (5-HT2AR) to produce profound alterations in perception, affect, and cognition. These alterations are often accompanied by experiences subjectively described as mystical or transcendental. Mystical-type experiences have been conceptualised as “quantum change experiences” (Yaden & Griffiths, 2020) – experiences that are profoundly personally meaningful or spiritually significant (Yaden & Griffiths, 2020). Psychedelic-occasioned mystical experiences are thought to facilitate new perspectives and insights, sudden emotional breakthroughs, and improved cognitive and affective flexibility (Carhart-Harris & Friston, 2019; Ko et al., 2022). These cognitive and affective changes appear to play a key role in the therapeutic potential of psychedelics. By facilitating psychological flexibility and perspective-shifting, psychedelic experiences may positively disrupt the rigid cognitive and behavioural patterns contributing to MDD maintenance (Carhart-Harris & Friston, 2019). Indeed, recent clinical trials have shown promising results for psychedelic drugs in the treatment of MDD (Carhart-Harris et al., 2016; Davis et al., 2021).

As interest in psychedelic-assisted therapy grows and research in this field expands, an important consideration is the potential interaction between psychedelics and commonly prescribed serotonergic antidepressants. This interaction is particularly relevant since many individuals seeking psychedelic experiences or therapies may be prescribed antidepressant medications. Understanding the pharmacological, psychological, and experiential aspects of combining psychedelics with SSRIs is crucial for ensuring patient safety and optimising therapeutic outcomes.

Pharmacological Considerations for SSRI-Psychedelic Interactions

While LSD and psilocybin share the central hallucinogenic mechanism of 5-HT2AR agonism, notable pharmacological differences exist between these agents that may influence their interaction profiles with SSRIs. LSD, an ergoline, demonstrates broader receptor activity, binding to dopamine D1-3 receptors and other 5-HT receptor subtypes (5-HT2B/2C, 5-HT1A), while psilocin, a tryptamine, exhibits more selective serotonergic activity with an additional inhibitory effect on SERT activity (Rickli et al., 2016). Comparative binding studies suggest that LSD may be a more potent 5-HT2AR agonist than psilocin (Rickli et al., 2016). Their distinct pharmacokinetic properties – with LSD producing effects lasting approximately 8 hours, compared to 5 hours for psilocybin (Ley et al., 2023) – may also influence the temporal profile of interactions with SSRIs. These pharmacological distinctions warrant careful consideration when evaluating safety profiles in combination with serotonergic agents.

As mentioned, both classical psychedelics and SSRI/SNRIs increase serotonergic transmission, albeit through different mechanisms. Excessive serotonin neurotransmission can induce serotonin syndrome/serotonin toxicity (ST), a potentially life-threatening condition characterised by dysregulated serotonergic neurotransmission (Malcolm & Thomas, 2021). This syndrome manifests through a complex interplay of neurotransmitter overstimulation, leading to symptoms across multiple physiological domains: neuromuscular abnormalities (e.g. tremor, hyperreflexia), autonomic nervous system hyperactivity (e.g., elevated heart rate, hyperthermia), and mental state alterations (e.g., agitation, delirium, confusion) (Sun-Edelstein et al., 2008). ST symptoms exist along a spectrum, ranging from mild serotonin-related side effects (e.g., insomnia, nausea, anxiety) to severe toxicity (e.g., cardiovascular destabilisation) requiring medical intervention (Gillman, 2004; Malcolm & Thomas, 2022). The pathophysiological mechanism involves excessive intrasynaptic serotonin concentrations, which overwhelm normal regulatory mechanisms and result in neurological and autonomic dysregulation (Scotton et al., 2019). Severe manifestations of ST are rare, with documented instances involving the concurrent ingestion of a monoamine oxidase inhibitor (MAOI) or serotonergic substance overdose (Brush et al., 2004; Culbertson et al., 2017; Pilgrim et al., 2012). Additionally, several case studies demonstrate that St John’s Wort (*Hypericum perforatum*) extracts can induce serotonin syndrome when administered in combination with serotonergic antidepressants (Lantz et al., 1999; Prost et al., 2000; Mannel, 2004). Mild ST symptoms can be difficult to distinguish from common psychedelic side-effects such as physical restlessness, nausea and insomnia, impacting the likelihood of their being accurately detected in contexts of concomitant psychedelic and serotonergic antidepressant administration.

Evidence for Psychedelic-Occasioned Therapeutic Effects

Psilocybin-assisted therapy has demonstrated rapid and sustained antidepressant effects in patients with treatment-resistant depression (Carhart-Harris et al., 2016; Davis et al., 2021). Similarly, LSD-assisted psychotherapy has shown potential in reducing anxiety associated with life-threatening illnesses (Gasser et al., 2014). The literature indicates that the long-term therapeutic benefits of psychedelics may depend on the quality of the peak subjective effects, as mediated by 5HT-2AR agonism (Griffiths et al., 2017; Jha et al., 2018; Yaden & Griffiths, 2020). Findings are mixed, however, with some studies suggesting that subjective effects are *not* necessary for enduring positive mood outcomes (Barbut Siva et al., 2024; Becker et al., 2022; Nautiyal & Yaden, 2022; Olson, 2021).

Griffiths et al. (2017) investigated psilocybin-occasioned mystical experiences in healthy participants across low and high-dose conditions. Stronger acute mystical-type experiences were observed in high-dose groups, with corresponding improvements at 6-month follow-up across several measures including interpersonal closeness, gratitude, and life meaning/purpose. Mystical experiences accounted for a significant proportion of the variance across all outcome measures, leading the researchers to conclude that psychedelic-occasioned mystical experience intensity predicts enduring positive trait-level changes in psychological and interpersonal functioning. Notably, 12% of participants in the low-dose group still rated their experiences as among the most spiritually significant of their lives

(Griffiths et al., 2017), suggesting that even reduced psychedelic effects can induce profound experiences, albeit less reliably than at higher doses.

Consistent with Griffiths et al.'s (2017) findings, Jha and colleagues (2018) demonstrated that psilocybin-induced oceanic boundlessness – a measure of mystical-type experiences – predicted sustained mental health improvements in individuals with treatment-resistant depression. Of note, oceanic boundlessness was a stronger predicter of reduced depressive symptoms than auditory or visual hallucinations. Together, these studies suggest that psilocybin-induced sustained positive mental health effects are determined, at least partially, by mystical experience intensity and quality (Griffiths et al., 2017; Jha et al., 2018).

Evidence for Therapeutic Effects with Concurrent SSRI Use

An important clinical consideration is that individuals taking serotonergic antidepressants may experience diminished psychedelic effects, a phenomenon referred to as "blunting" (Carhart-Harris & Nutt, 2017; Gukasyan et al., 2023). This observation had raised questions about whether the therapeutic benefits of psychedelics can be achieved in individuals maintaining SSRI treatment.

While some research suggests subjective psychedelic effects are required for enduring psychological benefits, several researchers argue that therapeutic outcomes can occur independently of acute subjective effects, despite a robust association between the two (Nautiyal & Yaden, 2022; Olson, 2021). Hesselgrave and colleagues (2021) showed that psilocybin's antidepressant-like effects in mice – measured by increased social reward and sucrose preference – persisted despite coadministration of the 5HT-2AR antagonist ketanserin (Nautiyal & Yaden, 2022). Since 5HT-2AR activation is believed to mediate the subjective effects of psychedelics in humans (Liechti et al., 2017), these preclinical findings suggest that some of psilocybin's therapeutic mechanisms may operate independently of 5-HT2AR activation.

Human studies similarly highlight a possible disconnect between the peak subjective experience and therapeutic effects of psychedelics. Barbut Siva and colleagues (2024) compared acute psychedelic effects and enduring therapeutic effects in individuals prescribed SSRIs versus individuals not taking psychiatric medications. Patients on SSRIs reported significantly reduced effects, consistent with earlier observational and case studies (Bonson et al., 1996; Strassman, 1992). However, both groups reported similar improvements in wellbeing and depressive symptoms at 4-weeks follow-up (Barbut Siva et al., 2024). This suggests that, while the acute subjective effects of psychedelics are diminished in individuals undergoing SSRI treatment, the therapeutic effects remain comparable to those not taking SSRIs. That is, although the acute subjective effects of psychedelics correlate with enduring therapeutic effects, this relationship appears to be mediated by factors beyond subjective experience intensity.

Becker et al. (2022) examined the subjective effects of psilocybin in healthy participants who received 14 days of pre-treatment with the SSRI escitalopram compared to those who underwent placebo pre-treatment. Contrasting several studies (Barbut Siva et al., 2024; Griffiths et al., 2017; Jha et al., 2018), the acute positive effects of psilocybin – including oceanic boundlessness, positive mood, and mystical experiences – were similar across escitalopram and placebo pretreatment groups (Becker et al., 2022). However, interestingly, escitalopram pretreatment significantly reduced negative acute effects – including adverse cardiovascular effects and anxiety – of psilocybin compared to placebo pretreatment. Since the acute positive effects of psilocybin were not affected by pre-treatment with escitalopram, Becker and colleagues (2022) suggested that the standard practice of discontinuing antidepressant treatment prior to psychedelic treatment may not be warranted.

Current Study

As psychedelics approach clinical implementation, understanding the safety implications of combining them with serotonergic antidepressants becomes increasingly critical. Current clinical protocols typically require SSRI discontinuation prior to psychedelic administration, often necessitating a challenging withdrawal period that may exacerbate underlying symptoms (Fava et al., 2015). However, recent findings suggesting maintained therapeutic benefit despite SSRI co-administration (Barbut Siva et al., 2024; Becker et al., 2022) challenge this conventional approach. Research on SSRI-psychedelic interactions remains limited and the full range of potential effects – both in terms of subjective experience and therapeutic outcomes – remains unclear, highlighting the need for systematic investigation of these interactions.

The present study investigated the subjective effects of psychedelic drugs (specifically, LSD and psilocybin) in individuals concurrently using SSRI/SNRIs, with an analytic focus on potential risks and adverse interaction effects. This research employed thematic analysis of retrospective, self-reported experiences published on Erowid.com, an online repository of user-generated psychoactive substance information. Thematic analysis was supplemented with content analysis to contextualise qualitative findings. The use of Erowid data in psychedelic research has precedent, with several studies utilising this resource to examine phenomenological aspects of psychedelic experiences (Hase et al., 2022; McCartney et al., 2023; Zamberlan et al., 2018). While user-reported data has inherent limitations regarding reliability, it offers unique advantages for examining naturalistic contexts and identifying emergent patterns that may not be captured in controlled settings. Furthermore, the anonymous nature of submissions may facilitate more candid reporting of adverse effects and risk management strategies. In systematically analysing these naturalistic accounts, we aimed to identify patterns of subjective effects, potential pharmacodynamic interactions, and perceived alterations in the psychedelic experience that may occur when these substances are combined. While acknowledging the limitations inherent in user-generated, non-controlled data, this exploratory study aimed to generate hypotheses and identify key phenomenological themes that may inform future controlled studies and contribute to our understanding of the complex interplay between classical psychedelics and serotonergic antidepressants at the experiential level.

The analysis was guided by three overarching research questions:

1. How do serotonergic antidepressants impact primary and emergent dimensions of psychedelic experiences?
2. What behavioural strategies do users employ to navigate SSRI/SNRI-psychedelic interactions?
3. What are the potential risks and adverse reactions associated with combining serotonergic antidepressants and psychedelics?

Method

Participants

The present study involved retrospective analysis of first-person, written experience reports, retrieved from the Erowid “experience vaults” database (2024). Erowid is a member-led educational organisation providing publicly accessible experiential accounts of psychoactive substance use. We did not extract age, gender, or ethnicity demographic information as these were (a) inconsistently reported across accounts, and (b) unrelated to the central research inquiry regarding adverse substance interaction effects. Psychedelic and antidepressant substance type and dose were manually extracted where reported, as was the duration of antidepressant use. Psychedelic dose strength was categorised according to established dose guidelines (Holze et al., 2021; MacCallum et al., 2022). For LSD, >25 µg (≤.5 tab) was considerate a low dose, 50 µg (1 tab) moderate, 100 µg (1.5-2 tabs) high, and ≥200 µg (≥2 tabs) very high (Holze et al., 2021; Holze et al., 2022). Due to the variability in the potency of LSD tabs in naturalistic contexts, these dose categories are approximations and should be interpreted cautiously. For psilocybin, ≤1.5 grams of dried *Psilocybe* mushroom (whole or powdered; ~≤15mg psilocin isolate) was considered a low dose, 2.5 grams (~25mg psilocin) moderate, 3.5 grams (~35mg psilocin isolate) high, and ≥5 (~≥50mg psilocin isolate) grams very high (MacCallum et al., 2022). However, psilocin concentrations vary from 0.01% to 2.4% by dry weight across *Psilocybe* species (Van Court et al., 2022; Beug & Bigwood, 1982), and species information was not extracted in the present study due to sparse reporting across participants. Use of the anonymised Erowid dataset was approved by the Victoria University of Wellington Human Ethics Committee (0000031472).

Procedure and Materials

Reviewed and edited Erowid reports from 2000 to 2024 were considered for inclusion. Reports were already anonymised with ID numbers. The “combinations” subsection of the LSD and psilocybin “experience vaults” database was systematically read and appraised against criterion for inclusion/exclusion. An additional search in the following sub-categories was conducted to ensure the collection of all relevant reports: “General”, “First Times”, “Retrospective/Summary”, “Difficult Experiences”, “Bad Trips”, “Train Wrecks & Trip Disasters”, “Glowing Experiences”, and “Mystical Experiences”. Inclusion criteria were (a) reports written in English, (b) psychedelic experiences involving LSD and/or psilocybin, (c) explicit or implied concurrent use of an SSRI, and (d) first-person accounts. Exclusion criteria were (a) reports over 2500 words, (b) concomitant use of benzodiazepines, amphetamines, or mood stabilisers (lithium, anticonvulsants, and antipsychotics), (c) psychedelic microdosing experiences, and (d) second-person experiences. Reports exceeding

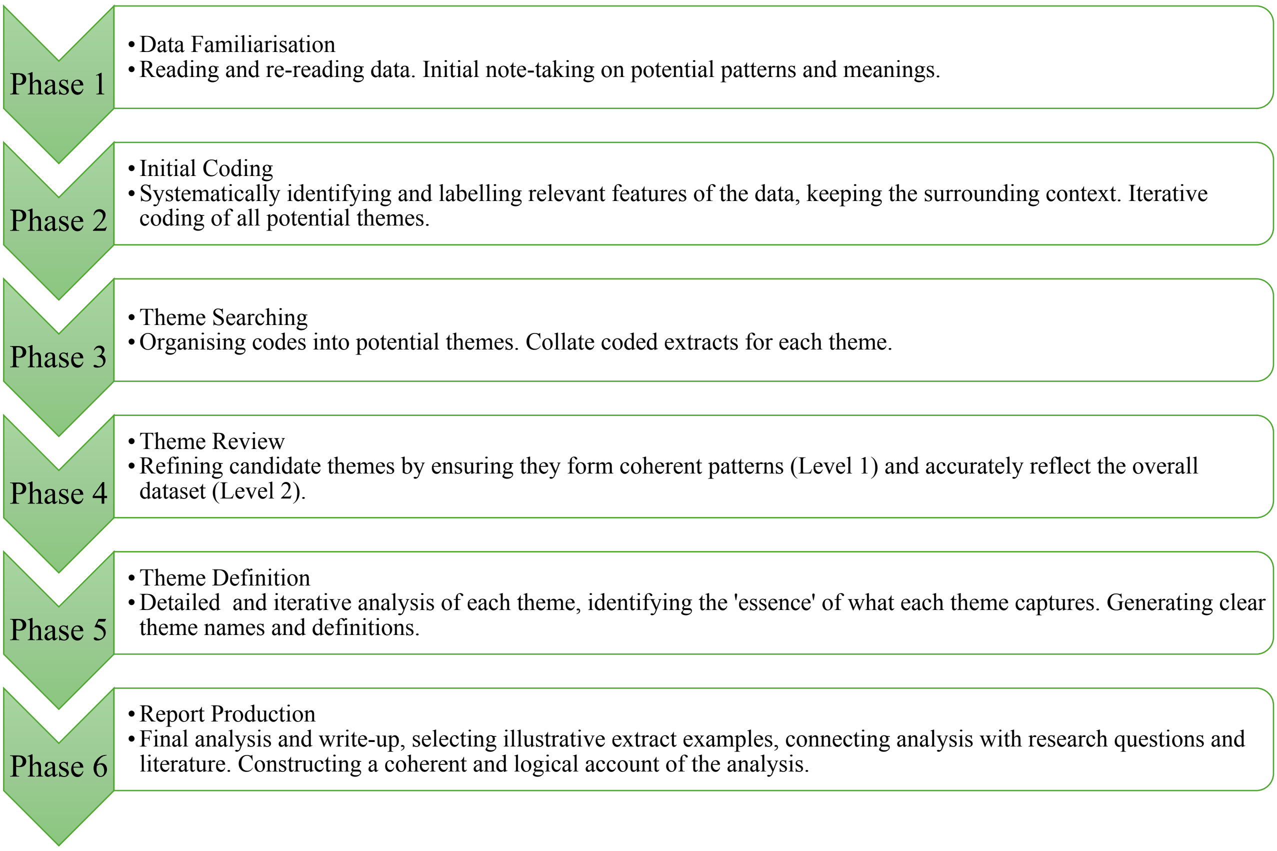
2500 words were excluded to maintain feasible scope for detailed manual thematic analysis of 41 reports. Several identified reports exceeded 4000 words, which would have significantly increased coding complexity without proportionate benefit to thematic saturation. During Phase 1 (data familiarisation), longer reports were observed to include tangential narrative details rather than additional content relevant to the research questions. The 2500-word limit allowed sufficient phenomenological detail while keeping the total dataset manageable for rigorous line-by-line analysis. This approach aligns with similar qualitative psychedelic research (e.g., McCartney et al., 2023), though our slightly lower threshold reflects our larger sample size. In contrast with some former research in this area (McCartney et al., 2023; Sapoznikow et al., 2019), it was determined that reports with concurrent cannabis use should be included to maximise ecological validity, given evidence that up to 40% of naturalistic psychedelic users report cannabis co-administration (Kuc et al., 2022; Licht et al., 2012). Additionally, research suggests that cannabis may dose-dependently modulate the subjective intensity of the psychedelic experience (Kuc et al., 2022); strategies that users employ to alter the acute effects of psychedelic substances are directly relevant to the current study's research questions regarding SSRI interaction management.

A total of 41 reports (20 psilocybin, 15 LSD, 6 both psilocybin and LSD) were identified as meeting the criteria; these were saved in PDF format on a password protected computer for analysis. Reports discussing both substances were counted in frequency calculations for each substance, resulting in 21 total reports involving LSD experiences and 26 involving psilocybin experiences. Reports were uploaded to NVivo 14 (2023), a data analysis software optimised for qualitative and mixed methods analysis.

Design

This study employed a qualitative research design, primarily utilising thematic analysis and supplemented by content analysis. The thematic analysis approach enabled in-depth examination of both manifest and latent meanings in the data, while frequency counts of specific content elements provided additional quantitative context to support the qualitative findings. This integrated method is consistent with that employed in psychedelic research by McCartney et al. (2021), Wood et al. (2024), and Robinson et al. (2024). A weak constructionist epistemological stance was deliberately assumed, whereby knowledge is understood to be subjectively constructed yet grounded in an underlying reality that can be meaningfully interpreted through systematic analysis (Crotty, 1998). Consistent with this epistemological framework, frequency counts are provided to contextualise patterns in the data rather than for statistical inference.

Figure 1



Braun & Clarke's (2006) Six Phases of Thematic Analysis

Note. Adapted from Braun and Clarke (2006).

Our thematic analysis combined traditional codebook development with Large Language Model (LLM) cross-checking. The primary analytical framework followed Braun and Clarke's (2006) six-phase approach to thematic analysis (see Figure 1), combining both inductive coding to capture emergent themes from the data and deductive coding guided by two established theoretical frameworks. First, we drew on the mystical experience framework developed through psychedelic research (Becker et al., 2022; Griffiths et al., 2017) – particularly the 30-item revised Mystical Experiences Questionnaire (MEQ30; Barrett et al., 2015; Maclean et al., 2012) – a validated measure which conceptualises mystical experiences through four factors: (1) unity, noetic quality, sacredness; (2) positive mood; (3) transcendence of time/space; (4) ineffability. Second, we utilised clinical diagnostic frameworks for serotonin syndrome (Malcolm & Thomas, 2021; Sun-Edelstein et al., 2008), which characterise the condition through spectral clusters of mental (e.g., agitation, confusion), autonomic (e.g., hyperthermia, tachycardia), and neuromuscular symptoms (e.g., tremor, hyperreflexia). While these frameworks informed our initial approach to the data, we aimed to balance theoretical sensitivity with openness to novel patterns emerging from participants' subjective experiences. Analysis was conducted through six phases.

Initial data familiarisation (Phase 1) began during data collection, with the primary researcher collecting and cleaning all written experience reports. Each report was read in full at least twice before coding began. During this phase, initial notes were made on potentially meaningful patterns and recurring ideas in the data. These preliminary observations were recorded in a research journal and discussed in regular meetings with the supervision team. Phase 2 involved systematic coding using NVivo 14. The coding process was iterative and followed predominantly inductive reasoning, allowing themes to emerge from participants' own accounts rather than fitting data to pre-existing frameworks. However, given the established literature on (a) serotonin toxicity (Malcolm & Thomas, 2021; Sun-Edelstein et al., 2008), and (b) mystical experiences in psychedelic research (Griffiths et al., 2017; Maclean et al., 2012), we remained sensitised to these concepts during coding. That is, while codes were primarily generated from the data (e.g., "I was concerned about any interactions I may experience, since I take Lexapro" ["Essere" Report ID: 98630], "I became very hot and realised I had somewhat of a fever" ["Mad Dog" Report ID: 8134]), we recognised when these aligned with documented features of serotonin toxicity (e.g., tremor, tachycardia) and/or mystical experiences (e.g., unity, transcendence of time/space). Each report was coded line-by-line, generating 56 distinct codes. Throughout the coding process, care was taken to avoid privileging mystical experience or serotonin toxicity concepts over other emerging patterns, ensuring that our analysis remained grounded in participants' lived experiences while acknowledging relevant theoretical constructs when they arose.

In Phase 3, codes were examined for potential patterns and relationships, with similar codes being clustered together into candidate themes. Initial thematic maps were constructed to visualise relationships between codes and potential themes. This process generated 11 candidate themes. In Phase 4, candidate themes and sub-themes were reviewed at two levels: first checking their coherence in relation to their coded extracts (Level 1), then assessing their validity in relation to the overall dataset (Level 2). This process resulted in 7 refined themes and 16 sub-themes that captured the central patterns in the data. Phase 5 involved detailed analysis of each theme to identify its 'essence' and its relationship to the research questions. Clear names and definitions were developed for each theme. Particular attention was paid to how themes related to each other and the overall narrative they illustrated about the data. Phase 6 consisted of selecting illustrative extracts for each theme and weaving together the analytic narrative. We aimed to construct a coherent narrative about the data while addressing the research questions and situating findings within existing literature. A detailed audit trail of coding and thematic decisions was maintained throughout the analysis process.

To enhance coding rigour and support reflexive analysis, the LLM Claude 3.5 Sonnet (Anthropic, 2024) was utilised as both a (a) comparative coding agent, and (b) critical codebook review partner. Claude 3.5 Sonnet was provided with the full dataset and researcher-developed codebook, including code definitions, inclusion/exclusion criteria, and exemplar quotes. The codebook included detailed operationalisations of both mystical experience dimensions and serotonin toxicity indicators based on established frameworks, ensuring theoretical consistency in the validation process. 8 reports (approximately 20.0% of the dataset) were then independently LLM-coded based on the provided codebook criteria, with reports selected to include equal representation of LSD (n = 4) and psilocybin (n = 4) experiences. The LLM applied the full hierarchical coding framework, providing an additional perspective on code application. Additionally, Claude 3.5 Sonnet systematically evaluated how effectively each code captured key phenomena present in the reports and identified possible gaps or redundancies in the coding framework. Code application differences between human and LLM coding were reviewed by the research team to prompt deeper engagement with the data and refine operational definitions where needed. Final coding decisions were made by the primary human researcher based on their contextual understanding of the data, treating the LLM as a dialogic partner rather than a statistical reliability check. This approach aligns with the study's weak constructionist framework, acknowledging that while multiple valid interpretations of the data are possible, the researcher's informed interpretation plays a central role in knowledge construction.

Results

Sample Characteristics

Participants (n=41) ranged from novice to experienced users of psychedelic substances. 27 participants (65.9%) had previous experience(s) with psychedelics, 10 (24.4%)

had no prior experience, and 4 (9.8%) did not specify. 15 reports (36.6%) described LSD experiences, 20 (48.8%) psilocybin experiences, and 6 reports (14.6%) discussed separate experiences with both substances. A range of serotonergic antidepressants were reported, with fluoxetine (Prozac) being the most common (34.1%, n=14), followed by sertraline (Zoloft; 22.0%, n=9), escitalopram (Lexapro; 12.2%, n=5) and venlafaxine (Effexor; 12.2%, n=5). Both antidepressant and psychedelic dosages ranged from low to above the maximum recommended dose. Antidepressant duration of use ranged from single use to 10 years (*M* ≈1.4 years), however several reports (31.7%, n=13) omitted this information. 14. See Appendix A for full participant characteristics.

Thematic Analysis Findings

Seven overarching themes were identified, forming interconnected patterns of psychedelic experience and response. Three themes captured core experiential aspects: Altered Acute Effects (*AAE*), Individual Response Variability (*IRV*), and Experiential Quality Changes (*EQC*). Two themes reflected user approaches to managing these experiential alterations: safety-oriented and risk management behaviours (Safety-Seeking Behaviours, *SSB*) and dose modification approaches (Dose-Response Adaptations, *DRA*). The Safety Profile and Adverse Events (*SPAE*) theme documented potential serotonin toxicity indicators and psychological crises, while Making Sense Through Comparison (*MSC*) captured the frameworks participants used to interpret their experiences through peer, substance, and expectation comparisons. Frequency counts are provided for all themes except *MSC*, as this theme captures interpretive frameworks rather than discrete experiential phenomena. See Appendix B for a summarised overview and description of the main themes, corresponding subthemes, and exemplar quotes.

Altered Acute Effects

AAE was a predominant theme across reports and describes the experience of diminished or altered psychedelic effects in comparison to previous non-SSRI experiences and/or peers' experiences in the same context. Three distinct sub-themes emerged: *diminished hallucinogenic effects*, *mind-body dissociation*, and *temporal disruption.*

Diminished Hallucinogenic Effects. Describes a reduction in the subjective intensity of visual hallucinations during the acute effects phase. Many participants (41.5%, n=17) explicitly described blunted or absent visual hallucinations (see Figure 2): "*I kept waiting for the intense and great visual I expected from acid but they never came*" ("kwmi" Report ID: 48587), "*By the peak I felt nothing other than a little different … but absolutely nothing visual*" ("maizeto" Report ID: 69871). However, several reports described experiences of sensory or perceptual distortion, despite these effects being subjectively experienced as sub-threshold hallucinations: "*Even though I was not hallucinating, the pictures in my minds eye were distorted … it seemed like the different sections of my face were coming together to meet as one*" ("Ease" Report ID: 53045), "*I began to see trails following my hands and legs as I moved them … stared into the mirror for a good five minutes and did not experience any sort of hallucination*" ("Spider" Report ID: 85489).

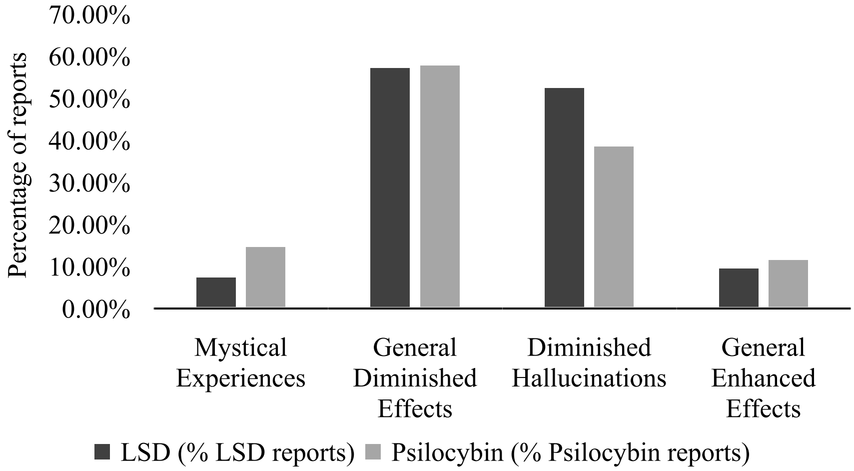
Mind-Body Dissociation. A small number of participants (9.8%, n=4) described a distinctive disconnection between somatic and psychological effects, where physical sensations associated with psychedelics were present but cognitive and "peak" effects were diminished: "*I felt like I was tripping physically for a few hours and couldn't sleep, but my state of mind was absolutely sober*" ("F" Report ID: 35488), "*I was only merely feeling the body high, waiting patiently for that loss of ego*" ("Afflatus" Report ID: 9291). This dissociation was particularly salient in reports contrasting current experiences with previous non-SSRI psychedelic use: "*The body feeling felt the same as it used to, but the hallucinations were diminished as well as the euphoria*" ("Tryptamine Fiend" Report ID:

79225). Some participants experienced this mind-body split as unsettling or disappointing: "*I basically had a mild body trip for a few hours but there were no visuals...and worst of all, there was no head trip whatsoever*" (“F” Report D: 35488).

Temporal Disruption. Describes reported alterations in the onset timing and duration of psychedelic effects compared to peer or pre-SSRI experiences. These were explicitly discussed in 19.5% (n=8) of reports. Some participants reported significantly shortened durations compared to previous non-SSRI experiences: “*A dose that would have resulted in a 3-4 hour experience 3 years ago at that time lasted about 1-1.5 hours*” (“The Ovoid Kid” Report ID: 115139), and compared to peers in the same context: “*I came down before everyone else, which I’m pretty sure was the effect of the Lexapro*” (“CT” Report ID: 52757). Conversely, other reports described unexpectedly prolonged effects: “*It lasted about 18-20 hours, when it was only supposed to last from 6-12 hours. I was shocked*” (“Belle” Report ID: 67323). Some participants described delayed onset of effects: “*Sixty minutes later I still felt nothing*” (“The Ovoid Kid” Report ID: 115139), “*It took me a while to come up, which I attribute to the SSRI*” (“CT” Report ID: 52757), while one participant described an unusually rapid onset: “*The come up was slightly dizzying. My thoughts started going into places and directions that don't exist when sober*” (“anonymous” Report ID: 107347).

Figure 2

Prevalence of Mystical Experiences and Acute Effect Alterations by Psychedelic Type



Individual Response Variability

This standalone theme captures the substantial variability in how individuals responded to psychedelic substances while taking serotonergic antidepressants (see Figure 2). More than half of participants (63.4%, n=26) reported diminished acute effects: "*Prozac had a definite dampening effect on the psilocybin*" ("Solipsist" Report ID: 94581), "*After I began the Paxil I found that the psychedelics lost their magic for me*" ("Tryptamine Fiend" Report ID: 79225). However, a notable proportion (26.8%, n=11) explicitly described undiminished (14.6%, n=6) or enhanced (12.2%, n=5) psychedelic effects: "*The more time that passed, the more intense my trip felt … it must be the daily doses of Fluoxetine*" ("Belle" Report ID: 67323), "*Some people say that Prozac reduces the effects of LSD … This was definitely not true for me. If anything, it enhanced the effects significantly*" ("ren" Report ID: 69000). Several participants reported an experiential distinction between the intensity and quality of psychedelic effects, describing positive appraisals of the psychedelic experience despite reduced effects: "*The overall quality of the experience was unchanged however, only the intensity was reduced, and only slightly*" ("desu desu desu" Report ID: 99310), "*I did* *hallucinate this time, but in a very light way … I wouldn't consider this a negative, however. Although I didn't see little gnomes sliding out of the ceiling on rainbows, I did have a life-changing experience*" ("listless" Report ID: 67759). One participant perceived reduced adverse effects with concurrent SSRI use compared to pre-SSRI experiences: "*The most important thing that I will say about using an SSRI with mushrooms is that it made it a lot harder to have a bad trip. Everything seemed perfect. Usually with shrooms I can get disoriented and paranoid or scared, but this trip was probably my favourite*" ("Kwmi" Report ID: 48586).

Experiential Quality Alterations

This theme describes alterations in the qualitative nature of the psychedelic experience independent of effect intensity. *EQA* is considered distinct from the intensity, or magnitude, of acute psychedelic effects (see Theme 1: Altered Acute Effects), instead consisting of affective changes, mystical or self-transcendent experiences, and alterations in the duration of effects. Two sub-themes were identified: *mystical experiences* and *changed emotional qualities*.

Mystical Experiences. Captures experiences characterised by profound alterations in consciousness, including experiences of unity, transcendence of space and time, and noesis. Mystical-type experiences emerged strongly in over one-fifth of the sample (22.0%, n=9), and were more common in psilocybin cases (14.6%, n=6) compared to LSD cases (7.3%, n=3) (see Figure 2). Several participants described experiences of profound interconnectedness and dissolution of typical subject-object boundaries: "*Memories, the past, the future and dreams all became one living experience*" ("Essere" Report ID: 98630), "*I had this strange feeling the entire trip of deja vu, like I had been here before … It was like I'd seen acid happen to me a thousand times before it ever actually did*" ("Ease" Report ID:

53045). Some reports detailed experiences of enhanced perception of meaning or essence: "*Then I noticed that objects took on a new vivacity, a sense that I was perceiving their 'true' form. All objects … seemed to be communicating their essence in a sort of emotional signal*" ("LinguaSiderea" Report ID: 50678). A subset of participants described experiences of profound personal or spiritual significance, often including sudden insights or new perspectives: "*I experienced a powerful 'coming home' type experience ... I was sobbing with joy ... The experience was powerfully emotional and cathartic*" ("N" Report ID: 95018), "*I felt like I had returned to a place inside myself that I had thought was lost … That was always inside me and I could always return to … A place where my true self dwells and maybe where god dwells as well. I felt so relieved and grateful to know that this place was there and accessible. My happiness was profound*" ("N" Report ID: 95018), "*I then realized that I wasn't alone in the Universe … I realized that my past life that I had constructed had been foolish. There had been so much suffering*" ("waner" Report ID: 13206).

Changed Emotional Qualities. Most participants described experiencing enhanced or altered emotions (70.7%, n=29), and several reports specifically comparing these emotional effects to previous pre-SSRI psychedelic experiences (17.1%, n=7). Changes in affective processing manifested in various ways, from heightened emotional sensitivity to imbuing environmental stimuli with emotion: "*Flowers opened to the sky in sheer joy. I stress that I was not experiencing hallucinations or even visual illusions ... What changed was the emotional valence attached to the environment*" ("LinguaSiderea" Report ID: 50678). Some participants described enhanced interpersonal emotional attunement: "*I was certainly more in tune with emotions (both mine and hers) than usual*" ("Solipsist" Report ID: 94581). A few reports noted positive emotional effects despite other aspects of the psychedelic experience being diminished: "*Although I didn't see little gnomes sliding out of the ceiling on rainbows, I did have a life-changing experience … for me, this raises the* *question not of how SSRIs affect the potency of mushrooms, but whether or not the interaction might produce a positive (healthful) experience*" ("listless" Report ID: 67759).

Safety-Seeking Behaviours

This theme captures approaches used to assess and manage potential risks related to combining psychedelics with serotonergic antidepressants in the absence of authoritative guidance. *SSB* comprises two sub-themes: *information gathering* and *risk management and safety planning*.

Information Gathering. Describes pre-experience methods of gathering and assessing information regarding the pharmacological interaction between psychedelic substances and SSRIs/SNRIs. Information-seeking behaviours ranged from extensive pre-experience research to minimal preparation, with approximately one-quarter of reports (24.4%, n=10) describing some attempt to understand potential interactions. Several participants reported difficulties locating reliable and useful interaction information: "*I resigned myself to a long trawl through the internet. I hardly found anything on the subject, a few dubious accounts but nothing much*" ("Sparky" Report ID: 24962), "*I was concerned about any interactions I may experience ... I searched the web, but, to my dismay, only found a handful of posts*" ("Essere" Report ID: 98630). Some reports described a reliance on word-of-mouth knowledge or standalone online retrospective accounts: "*I was obviously concerned about the possibility of a dangerous interaction. When I asked a mate of mine who does biochem about it, he suggested that an interaction was unlikely*" ("Sparky" Report ID: 24962), "*I had read a trip report where someone had taken 25mg and 1 tab and had been fine … so I assumed it was safe*" ("Raizan" Report ID: 111001). The interaction information obtained by participants was incorrectly interpreted in some instances, leading to erroneous theories or assumptions regarding possible drug interaction effects: "*I know that psychedelics* *(especially mushrooms) cause a rush of serotonin to the brain. My hypothesis was that the SSRI would lengthen or intensify a trip by inhibiting the reuptake of the released serotonin*" ("kwmi" Report ID: 48587), "*I remembered hearing that MAOIs also add to the effect, so I dug up a bottle of Prozac I had around from years ago*" ("JoeShmo" Report ID: 15247).

Risk Management and Safety Planning. Describes measures taken – prior to or during the psychedelic experience – to negate or reduce the risks associated with psychedelic use. A subset of reports (17.1%, n=7) explicitly described premeditated safety planning or risk mitigation strategies. Common strategies included maintaining access to emergency support, arranging trusted trip sitters, and selecting familiar settings: "*We decided to stay in a nice safe place (i.e. our home) with no one we felt unfamiliar or insecure with around to limit the chances of a bad trip*" ("Spider" Report ID: 85489), "*It certainly helped to be with my boyfriend for the day. He knows what to do if I ever need help … This made me feel incredibly safe so if things started to go south, I wouldn't be lost in my mind*" ("Lady" Report ID: 109504). The described preparatory measures were applicable to general psychedelic use, rather than specific to the interaction between psychedelics and serotonergic antidepressants.

Dose-Response Adaptations

DRA emerged as a strong, consistent theme across reports. This theme describes strategic modifications to psychedelic and/or serotonergic antidepressant dosing patterns to achieve the desired psychedelic effects. Four sub-themes emerged: *intentional dose compensation*, *unplanned redosing*, *antidepressant tapering*, and *potentiation strategies*.

Intentional Dose Compensation. Refers to the pre-experience decision to consume a high psychedelic dose to counter the expected blunting effects of coadministration with serotonergic antidepressants. 19.5% (n=8) of reports described intentional psychedelic dose compensation. Several participants described deliberately taking larger initial doses based on previous experiences or expectations of diminished effects: "*5 is the highest dose of mushrooms I have done, which I took with the expectation that I would be getting a three gram dose … if I accounted for the inhibiting effect of Lexapro*" ("followthruthefloor" Report ID: 115228). Some participants reported consuming extremely high doses in attempts to overcome perceived SSRI/SNRI blunting: "*I was craving a stronger experience, something I could really call a breakthrough … I decided to take four tabs*" ("anonymous" Report ID: 107347), "*After reading up on the interactions between psilocin/psilocybin and various SSRIs, I resolved to take the remaining 600mL … all at once. Surely, I thought, visuals would come from such a dose*" ("The Ovoid Kid" Report ID: 115139).

Unplanned Redosing. Describes the ad-hoc decision to consume additional psychedelic doses during the experience when initial doses produced weaker than expected effects. 22.0% (n=9) of participants reported ad-hoc psychedelic redosing. This typically occurred as an improvised response to subjectively dissatisfying peak effects: "*After the first hit I felt absolutely nothing. About two hours after taking the first hit I took two more…*" ("ZoloftShroomer" Report ID: 70865). Some participants described multiple rounds of redosing: "*At around the 45 min mark, and still at baseline, I decided to take 10 more grams worth of the brew... Sixty minutes later I still felt nothing, so I took 20g more*" ("The Ovoid Kid" Report ID: 115139). Reports suggested that this strategy was often ineffective: "*I decide to take another trip hoping that I can have a better one than the night before. I take two blotter tabs … Nothing happening at all. I take another blotter tab … Didn't trip, had a headache the whole time pretty much*" ("420time" Report ID: 70560).

Antidepressant Tapering. Captures the intentional reduction or temporary cessation of serotonergic antidepressant medication to enhance psychedelic effects. 17.1% (n=7) or reports explicitly described serotonergic antidepressant tapering or discontinuation prior to psychedelic use. Some participants reported skipping single doses: "*I have missed my daily* *dose of citalopram 40mg hoping this will help the LSD do its thing*" ("420time" Report ID: 70560), while others described more extended periods of abstinence: "*I decided to not take my medicine for five days before I dosed...*" ("Matthew 'Holden' Ulm" Report ID: 42405). The perceived success of this strategy varied, with some participants reporting restored psychedelic effects: "*HALLELUJAH!! I finally had my first REAL trip in almost a year (about the time I began my medication)*" ("Matthew 'Holden' Ulm" Report ID: 42405), while others found minimal benefit: "*Six weeks after emancipating myself from the bonds of the chemical handcuffs we call Lexapro, I enthusiastically attempted to try mushrooms again. The letdown was visceral, no change whatsoever*" ("The Ovoid Kid" Report ID: 115139). One participant described reemerging depression symptoms due to pre-experience SSRI discontinuation: "*I feel the effects of my seasonal kicking back in as I have gone cold turkey the last 3 days preparing for my shroom trip*" ("Ashton" Report ID: 113828).

Potentiation Strategies. Describes the use of additional substances or environmental factors to enhance psychedelic effects; these were explicitly discussed by 4 participants (9.8%). One report described using cannabis in attempt to intensify or instigate the experience: "*I smoked some high-grade marijuana to see if I could catapult myself into the divine. It did do the job for around a half of an hour*" ("Afflatus" Report ID: 9291), while one participant implied prior effective use of this potentiation method: "*I bemoaned not having brought any weed, since this has a tendency to kickstart a trip that is lacking or taking a while to get started*" ("followthruthefloor" Report ID: 115228). Other potentiation strategies included the use of monoamine oxidase inhibitors (MAOIs): "*Two grams of Syrian rue seeds marginally increased both the duration and magnitude of the experience*" ("The Ovoid Kid" Report ID: 115139), nitrous oxide: "*B and I used a couple of capsules of nitrous, which deepened the trip*" ("anonymous" Report ID: 107347), and environmental modifications such as music: "*I was tripping a little bit. I took advantage if this and put on some psych rock, to try and help myself get more into the headspace*" ("followthruthefloor" Report ID: 115228).

Safety Profile and Adverse Events

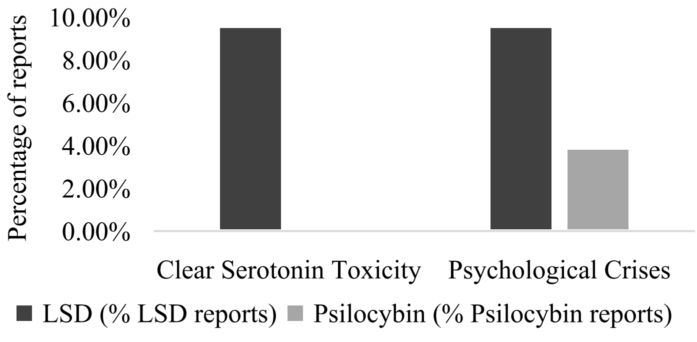
This theme describes the range and severity of adverse effects reported when combining psychedelics with serotonergic antidepressants. Two key subthemes emerged: serotonin toxicity risk and psychological crises.

Serotonin Toxicity Risk. Captures experiences potentially indicative of serotonin toxicity (ST), as defined by concurrent autonomic, neuromuscular, and mental status changes (Malcolm & Thomas, 2021). In 4.9% of cases (n=2), participants reported clear ST indicators across all three symptom domains – autonomic, neuromuscular, and mental status changes. Both cases of clear ST involved LSD (9.5% of LSD reports) (see Figure 3). 1 case (2.4%) of possible serotonin syndrome – wherein changes were observed in autonomic and psychological, but not neuromuscular, symptom domains – occurred with psilocybin (3.8% of psilocybin cases). Characteristic descriptions included severe physiological dysregulation combining autonomic and neuromuscular symptoms. One participant described progressive temperature dysregulation and motor impairment: "*I began staggering about and trembling rather severely. I felt very hot and sweaty … sweating so profusely that it was dripping off of my body very quickly. I felt very sick and weak. After a while I felt very cold ... shivering uncontrollably, and felt gripped by hypothermia"* ("Mad Dog" Report ID: 8134). The same participant later developed respiratory difficulties: "*I had trouble breathing the whole night, and often found myself taking huge breaths and gasping to get more and more oxygen in*". Notably, this participant ("Mad Dog" Report ID: 8134) was not prescribed a serotonergic antidepressant at the time of their psychedelic experience and consumed a sertraline dose that greatly exceeded standard dosage guidelines. Another case demonstrated concurrent autonomic and mental status changes: "*I was struggling to breathe and my mouth was so dry, I was sweating so much and I never had felt so thirsty in my life. I was struggling to swallow and breathe and I couldn't stop crying*" ("Raizan" Report ID: 111001). This case progressed to require emergency medical intervention due to cardiovascular symptoms: "*The operator heard me scream 'MY HEART IS BURNING IT HURTS SO BAD' … My boyfriend quickly had his hand on my pulse and was checking it at the operator's command … when he told her 70 beats in 15 seconds* [indicating a heart rate of 280 BPM] … *The ambulance had changed from a 45-minute wait to an immediate response".* Medical management included anxiolytic treatment: "*I got put in a room with a door for privacy and she came and gave me some diazepam*" ("Raizan" Report ID: 111001).

Psychological Crises. Describes experiences of overwhelming psychological distress that exceeded typical challenging psychedelic experiences in both intensity and nature. 3 reports (7.3%) met this criterion, 2 of which involved LSD (9.5% of LSD cases), and 1 psilocybin (3.8% of psilocybin cases) (see Figure 3). Participants described states of severe psychological crisis marked by intense paranoid ideation and existential terror: "*All I knew was that I was in big fucking trouble. They, with a capital T, were after me and if they caught me my life was effectively over. This crushing feeling of what-have-I-done dominated my consciousness... The apartment was no longer a safe place*" ("anonymous" Report ID: 107347). Some participants reported overwhelming psychological fragmentation: "*Have you ever imagined what it is like to be insane? Ever imagined what it's like to be alone, SO ALONE, trapped in your mind with every sight and sound and image and thought and emotion SCREAMING in your mind, almost compacting itself and exploding into one idea and sensation that destroys reality*" ("JoeShmo" Report ID: 15247). The most severe cases resulted in actively dangerous behaviour requiring emergency intervention, including one documented suicide attempt following intense psychological distress: "*Despaired, I started* *ascending the stairs. I climbed to the roof and jumped. I crashed on concrete after falling about 65 feet... I survived the fall but spent nearly three months hospitalized*" ("anonymous" Report ID: 107347). Some reports described psychological crises occurring alongside potential serotonin toxicity symptoms: "*I couldn't shake the fear of dying and the fact that my time was coming for me, every inch of my spirit and soul screamed out to live... I have had a history of struggling with panic attacks but they stopped for a year or so, only to come back during this trip with the worst one of my life*" ("Raizan" Report ID: 111001). One participant explicitly attributed their psychological crisis to the interaction between psychedelics and serotonergic antidepressants: "*This mind fuck could have been the product of any number of factors, but I believe the Prozac + Shrooms was the bad move*" ("JoeShmo" Report ID: 15247). Of note, this case ("JoeShmo" Report ID: 15247) reported that they had not taken a serotonergic antidepressant (fluoxetine) for several years prior to their psychedelic experience – consumption was driven by the erroneous belief that fluoxetine is a MAOI that may enhance the intensity of the psychedelic effects.

Figure 3

Prevalence of Adverse Effects by Psychedelic Type



Making Sense Through Comparison

This theme describes how participants used various comparative frameworks to understand and interpret their experiences. Analysis indicated that individuals actively sought reference points – including peer experiences and previous trips without concomitant serotonergic antidepressant use – to make sense of their SSRI/SNRI-influenced psychedelic experiences. Three distinct subthemes emerged: *social comparisons*, *cross-substance comparisons*, and *expectations versus reality*.

Social Comparisons. Encompasses participants' tendency to evaluate their experiences relative to non-serotonergic antidepressant using peers in sharing settings. Many participants used peer comparisons as a primary method for gauging how SSRI/SNRIs impacted their psychedelic response: "*My friends all ate the same amount and were in a completely different state of mind then I was. They were all having a complete trip but nothing happened to me*" ("dly2588" Report ID: 64072), "*Friends who I regularly tripped with were having a great time, saying this was the best acid that had come around in a while, and I was barely feeling anything*" ("F" Report ID: 35488). Participants typically described situations where peers reported strong effects while they experience minimal changes: "*I ended up taking about twice as much as my friends, although it seemed that my experience was markedly less intense than theirs were*" ("Bran Man" Report ID: 72821).

Cross-Substance Comparisons. Describes how participants evaluated and contrasted the effects of different psychedelic substances when combined with serotonergic antidepressants. A small subset of accounts (7.3%, n=3) described differential blunting effects between LSD and psilocybin. Two participants reported greater blunting with LSD compared to psilocybin: "*Since starting on Effexor, I have found that the effects of shrooms, and especially acid, are greatly diminished*" ("F" Report ID: 35488), "*SSRI + LSD - subdued trip. SSRI + Shrooms - Shorter but more intense trip*" ("kwmi" Report ID: 48587), while one report described stronger blunting with psilocybin: "*A complete letdown … This same scenario played out every time I ate mushrooms while taking Lexapro. LSD, on the other hand, worked just like it always had before Lexapro. It actually seemed marginally stronger* *than it was prior to Lexapro*" ("The Ovoid Kid" Report ID: 115139). One participant perceived the intensity of the acute psychedelic effects as unaffected by serotonergic antidepressant (sub)type: "*I was taking these three antidepressants … About 6 months for Effexor, 6 months for Prozac, and a year for Zoloft. With each … I reached the so-called 'high dose'. I reached the conclusion that with each antidepressant the results from mixing other chemicals stayed the same*" ("K" Report ID: 8334).

Expectations versus Reality. Captures the disparity between anticipated and actual effects of combining psychedelics with serotonergic antidepressants. Some participants approached the experience with specific expectations based on prior research or peer accounts: "*So when he said he tripped balls, I fully believed him and anticipated that I would do the same ... I was not under the impression that the effects of tryptamines would become diminished*" ("maizeto" Report ID: 69871). The mismatch between expectations and reality often led to disappointment: "*The trip was nowhere near as intense as I'd hoped. I love crazy intense experiences, and at times the trip was wild, but I was far too in control of the experience for it to fulfill my hopes*" ("Ease" Report ID: 53045), "*My experiences with MDMA told me that the SSRIs would decrease the effects of those drugs, but I didn't think it would apply to mushrooms … I was super excited … I think the Prozac had a definite dampening effect on the psilocybin. It's too bad*" ("Solipsist" Report ID: 94581). However, some participants reported positive experiences despite being aware of accounts of diminished effects: "*I had done a bunch of research beforehand … All I turned up was that my Lexapro was probably going to lessen the effects of acid or shrooms ... Overall, I really enjoyed my acid trip*" ("CT" Report ID: 52757), "*I had been warned that the psilocybin might not be as effective … I did have a life changing experience. My social anxiety disappeared completely … Ultimately, I felt the most relaxed (socially and mentally) I have ever felt in my life*" ("listless" Report ID: 67759).

Discussion

The present study examined the phenomenological and safety aspects of combining psychedelic substances (LSD and psilocybin) with serotonergic antidepressants through qualitative analysis of naturalistic experience reports. Our analysis revealed four key, interrelated findings: (1) preservation of mystical-type experiences despite sensory/perceptual dampening, (2) marked individual response variability, (3) differential safety profiles between psychedelic substances, and (4) concerning patterns in user approaches to managing these interactions. Here we examine each finding's implications for both clinical practice and harm reduction.

Persistence of Mystical-Type Effects

A key finding was the apparent dissociation between different aspects of the psychedelic experience. While visual/sensory effects were commonly reported as diminished, 22.0% (n=9) reported profound mystical-type experiences characterised by feelings of unity, transcendence of space/time, and noetic quality. Mystical experiences were more common for psilocybin cases (14.6%, n=6) than LSD cases (7.3%, n=3), consistent with controlled studies indicating higher rates of mystical experiences for psilocybin (up to 67.0%) compared to LSD (12.5%) in non-SSRI populations (Barrett et al., 2015; Barrett & Griffiths, 2018). Among these reports, 12.2% (n=5) explicitly described this pattern of reduced sensory effects coinciding with intact mystical/emotional components. This observation has important implications for understanding the therapeutic mechanism of psychedelics, as mystical experiences have been shown to mediate the enduring beneficial psychological effects of psychedelics (Griffiths et al., 2017; Jha et al., 2018; Roseman et al., 2018).

The preservation of mystical experiences despite reduced sensory effects in some participants may help explain the persistence of therapeutic effects even when acute effects are attenuated by SSRIs, as observed by Barbut Siva et al. (2024) and predicted by preclinical models (Hesselgrave et al., 2021; Nautiyal & Yaden, 2022). This finding can be interpreted through the lens of the Relaxed Beliefs Under Psychedelics (REBUS) model (Carhart-Harris & Friston, 2019). The REBUS framework proposes that psychedelics reduce the brain's reliance on prior expectations and beliefs, enhancing sensitivity to bottom-up information flow which promotes a greater diversity of cortical states. According to this model, this reduction in top-down control may facilitate the emergence of mystical-type experiences by loosening the constraints typically imposed by higher-order neural systems on emotion, cognition, and perception. The relaxation of higher-level priors may have facilitated the emergence of mystical-type experiences in our sample. This effect likely occurred through changes in the Default Mode Network (DMN), a neural network crucially implicated in self-referential thought and meaning-making (Carhart-Harris et al., 2014). However, this interpretation is speculative and requires further investigation to establish a causal relationship.

Interestingly, a smaller subset of participants (9.8%, n=4) described an opposite pattern – intact sensory effects with absent cognitive/emotional effects. These contradictory findings suggest that serotonergic antidepressants may differentially impact various aspects of the psychedelic experience rather than uniformly attenuating them. This variable response pattern may reflect individual differences in 5-HT2AR distribution and function across cortical regions (Erritzoe et al., 2019), as well as differential effects of chronic versus acute SSRI use on affective processing (Burghardt & Bauer, 2013).

These findings highlight the complex neural mechanisms underlying psychedelic effects and suggest their therapeutic potential may be independent of acute sensory alterations. Rather, their ability to enhance cognitive flexibility and update higher-order beliefs may be crucial for their therapeutic efficacy. Further research is needed to clarify the relationship between these dissociated effects and antidepressant outcomes.

Individual Response Variability

A notable finding was the substantial individual variability in how SSRI/SNRIs modified the psychedelic experience. While most participants (63.4%, n=26) reported diminished psychedelic effects compared to their previous experiences without antidepressants and peer experiences in shared environments, a substantial proportion (26.8%, n=11) explicitly described either undiminished (14.6%, n=6) or enhanced (12.2%, n=5) effects. This heterogeneity challenges clinical findings suggesting that acute psychedelic effects are largely unaffected by concurrent SSRI use (Becker et al., 2022). Our results align more closely with Barbut Siva and colleagues' (2024) finding of generally reduced acute effects in SSRI-treated individuals.

The observed variability likely reflects multiple interacting factors, including individual differences in drug metabolism and receptor polymorphisms (Vizeli et al., 2021; Yohn et al., 2017). CYP2D6 polymorphisms result in various metaboliser phenotypes, including poor metabolisers (5-10% in Caucasians, about 1% in Asians) and ultra-rapid metabolisers (3-5% of the population), which directly affects drug bioavailability (Koopmans et al., 2021; Vizeli et al., 2021). Many SSRIs inhibit CYP2D6, potentially affecting the metabolism of other CYP2D6 substrates (Austin-Zimmerman et al., 2021; Hemeryck & Belpaire, 2002). Psychological factors, especially expectations shaped by prior psychedelic experiences, likely contribute to response variability (Colloca et al., 2023). Environmental context, including setting and social dynamics, may also play an important role (Haijen et al., 2018). One speculative mechanism is that differences in serotonin receptor adaptation to chronic SSRI treatment, such as the degree of 5-HT2AR downregulation, may influence how these medications modify psychedelic effects (Yohn et al., 2017). Further research using controlled designs is needed to disentangle these potential contributing factors.

User Risk Management and Effect Modulation Strategies

Our analysis revealed concerning patterns in how users attempted to manage and modulate the SSRI-psychedelic interaction. Three primary high-risk behaviours emerged: psychedelic dose escalation, unplanned redosing, and unsupervised antidepressant discontinuation. Many participants (22.0%, n=9) engaged in ad-hoc psychedelic redosing when initial doses produced weaker than expected effects, often making improvised dosing decisions based on immediate subjective effects rather than safety considerations. This strategy was frequently ineffective and potentially unsafe, with several participants reporting adverse effects such as prolonged headaches following multiple redoses.

A substantial proportion of participants (17.1%, n=7) described unsupervised tapering or complete discontinuation of serotonergic antidepressant prior to psychedelic use. While some reports described partial or complete restoration of psychedelic effects following antidepressant discontinuation, others reported adverse effects including reemerging depression symptoms. This finding is particularly concerning given the risks associated with abrupt SSRI/SNRI discontinuation – namely serotonin discontinuation syndrome (characterised by mild physical symptoms that begin within days of tapering/discontinuation) and reemerging depression (characterised by the return of core depression symptoms, typically developing within weeks of tapering/discontinuation) (Gabriel & Sharma, 2017). Indeed, one participant (“Ashton” Report ID: 11382) reported reemerging depressive symptoms after only three days of SSRI discontinuation. However, as core depressive symptoms generally take weeks to emerge following discontinuation, this short timeframe may represent serotonin discontinuation syndrome or a placebo response rather than true depression recurrence.

These high-risk behaviours appeared to be driven by several factors. Only one-quarter of participants (24.4%, n=10) reported attempting to research potential interactions before combining substances. Those who did seek information described difficulties accessing reliable sources, often relying on single user reports or word-of-mouth knowledge. Most concerningly, some participants dangerously misinterpreted information about drug interactions. One participant consumed an extremely high sertraline dose based on an incorrect belief that higher quantities of serotonergic antidepressants would enhance the effects of psychedelics. This led to the emergence of symptoms consistent with serotonin syndrome.

Additionally, social comparison processes appeared to influence dosing decisions, with many participants escalating doses when perceiving attenuated effects compared to peers in shared settings, despite limited understanding of how serotonergic antidepressants could modify both the subjective experience and safety risks.

Several participants described using additional substances to potentiate psychedelic effects. While cannabis temporarily enhanced effects, this enhancement was brief: "I smoked some high-grade marijuana to see if I could catapult myself into the divine. It did do the job for around a half of an hour" ("Afflatus" Report ID: 9291). Notably, one case involved nitrous oxide use culminating in a severe psychological crisis and suicide attempt, highlighting the risks of combining multiple consciousness-altering substances.

The widespread lack of research into pre-experience interactions suggests many users may be psychologically unprepared for attenuated psychedelic effects when taking serotonergic antidepressants. This expectation-reality mismatch could impact users' ability to recognise and engage with subtle mystical or therapeutic aspects of the experience, particularly given evidence that mystical effects may persist even when sensory effects are reduced. The limited consideration of potential interactions also raises safety concerns, as users may not anticipate the physiological risks of combining serotonergic compounds.

These findings highlight critical gaps in drug education and harm reduction resources for individuals combining these substances in naturalistic settings. The limited availability of reliable SSRI-psychedelic interaction information, combined with poor understanding of pharmacological risks, may contribute to potentially dangerous dose escalation and substance combination practices. Our findings reinforce the pressing need for accessible and accurate information about psychedelic-antidepressant interactions to support safer decision-making in naturalistic contexts.

Differential Safety Profile by Psychedelic Type

While the interaction between psychedelics and serotonergic antidepressants was generally well tolerated, observed risk profiles differed for LSD versus psilocybin. Clear serotonin toxicity indicators occurred exclusively in LSD experiences (9.5% of LSD reports). Psychological crises were also more prevalent with LSD (9.5%) compared to psilocybin reports (3.8%). This pattern may reflect distinctions in the pharmacological properties of these substances (Zamberlan et al., 2018).

Firstly, LSD demonstrates broader receptor activity than psilocybin, acting as a partial agonist at multiple serotonin (5-HT2A-C) and dopamine receptors (D1-3) (Rickli et al., 2016). Conversely, psilocybin primarily targets 5-HT2AR, with less pronounced activity at 5-HT2B and 5-HT2C receptors. Further, LSD exhibits greater binding affinity to 5-HT2AR than psilocybin (Rickli et al., 2016), which may exacerbate serotonin-related side effects when combined with SSRI/SNRIs.

Secondly, Shinozuka et al. (2024) found that LSD induces greater inositol 1,4,5-triphosphate (IP3) formation compared to psilocin, suggesting stronger activation of the Gq/11 protein-coupled signalling pathway downstream of 5-HT2AR activation. The IP3 pathway plays a central role in mobilising intracellular calcium, as IP3 binds to IP3 receptors in the endoplasmic reticulum, initiating the release of stored calcium ions into the cytoplasm (Nichols, 2016). In contrast, psilocybin demonstrates preferential activation of phospholipase A (PLA) pathways over IP3, which may confer a lower risk of excessive serotonergic signalling with concomitant SSRI use (Sakai et al., 2024). LSD's greater relative impact on IP3 production may result in higher intracellular calcium levels and thus enhanced neuronal excitability and downstream signalling cascades, heightening the risk of serotonergic overstimulation.

Together, these studies (Nichols et al., 2016; Rickli et al., 2016; Shinozuka et al., 2024) suggest that the SSRI-psychedelic interaction likely involves mechanisms beyond simple competition at 5-HT2AR, potentially including differential effects on various receptor subtypes and downstream signalling pathways (Cameron et al., 2023; Kwan et al., 2022). These pharmacological differences may explain our observation of substance-dependent adverse event patterns and suggest that psilocybin offers a more favourable risk-benefit profile than LSD for therapeutic applications involving individuals taking serotonergic antidepressants.

Clinical Implications and Future Directions

The present study has several important clinical applications. Firstly, the differential rates of adverse effects with LSD versus psilocybin suggests a superior safety profile for psilocybin than for LSD when combined with serotonergic antidepressants. This finding could directly inform both clinical trial design and harm reduction approaches.

Secondly, our findings challenge the necessity of SSRI discontinuation protocols in psychedelic-assisted therapy. As mentioned, serotonergic antidepressant discontinuation carries risks of both serotonin discontinuation syndrome and symptom re-emergence (Gabriel & Sharma, 2017). In designing clinical psychedelic trials, researchers must balance these discontinuation risks against the risk of SSRIs confounding results. Gukasyan et al. (2023) found that SSRI-related dampening of psychedelic effects can persist for up to 3-months post-discontinuation. However, since typical protocols only require 4-5 weeks of SSRI tapering and washout (Carhart-Harris et al., 2016; Davis et al., 2021), current strategies may be insufficient to prevent confounded effects anyway. Alternative approaches such as controlled dose adjustment (Erritzoe et al., 2024) and differential dosing based on CYP2D6 metaboliser status may be more appropriate, though additional research is needed to establish their safety and efficacy.

Finally, substantial individual variability in psychedelic experiences emphasises the need for personalised clinical approaches, including careful consideration of patient expectations and psychological readiness. Given the established influence of 'set' (mindset and expectations) on psychedelic experiences (Haijen et al., 2018), clinicians should guide client expectations about the magnitude and quality of the psychedelic effects they may experience with concurrent SSRI/SNRI use. Rather than focusing solely on potential reductions in visual or sensory effects, clinicians should emphasise that meaningful therapeutic and mystical experiences remain possible, as demonstrated by participants who described experiences of profound personal significance or psychological insight even when reporting minimal hallucinogenic effects. Preparedness for this pattern of effects – potentially reduced sensory experiences alongside preserved mystical/therapeutic components – may facilitate psychological openness to the emergence of clinically relevant phenomena.

Our findings have several limitations. The use of Erowid reports as the sole data source introduces potential recall and sampling biases, as individuals who submit reports may not be representative of the broader population combining these substances. Our small sample size precluded meaningful analysis of dose-dependent effects or comparisons across specific serotonergic antidepressant types, and the naturalistic setting meant that substance purity and exact dosage could not be verified. Moreover, some of our reported percentages, particularly regarding adverse events and differential substance effects, are based on limited numbers ($n < 5$) and should be interpreted with appropriate caution. The unstandardised reporting format and limited demographic information restricted our ability to systematically compare experiences across reports or analyse demographic factors that might influence outcomes. Additionally, we were unable to verify participants' reported diagnoses or medication regimens, and there may be reporting bias towards particularly positive or negative experiences, potentially skewing the representation of typical interactions. Despite these limitations, we argue that the detailed phenomenological accounts provide valuable insights into the subjective experience of combining psychedelics with SSRI/SNRIs and generate valuable hypotheses for future controlled studies.

Future research should prioritise controlled studies examining the differential safety profiles of various psychedelic-SSRI/SNRI combinations. In particular, the mechanisms underlying reduced risk of serotonin toxicity and adverse psychological events with psilocybin compared to LSD warrant controlled investigation. Additionally, investigation of individual difference factors contributing to response variability would support development of personalised therapeutic approaches, such as CYP2D6 phenotyping and psychological readiness assessments to inform optimal dosing strategies. Furthermore, clinical trials directly comparing the efficacy of psychedelic-assisted therapy with and without serotonergic antidepressant discontinuation would help establish evidence-based protocols. Finally, standardised guidelines for identifying and managing adverse physiological and psychological events in individuals combining classical psychedelics with serotonergic antidepressants would improve dose measurement, clinical practice, and harm reduction efforts.

In summary, this qualitative analysis of naturalistic psychedelic experiences revealed complex interactions between serotonergic antidepressants and psychedelics, characterised by variable relationships between mystical and sensory effects, substantial individual response variability, differential substance safety profiles, and high-risk user behaviours. These findings underscore the urgent need for accessible harm reduction resources, controlled research into underlying mechanisms and individual differences, and evidence-based clinical guidelines to optimise therapeutic outcomes and mitigate risks for individuals combining psychedelics with serotonergic antidepressants.

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Appendix A

Participant Demographics Table

Participant ID	Antidepressant type	Antidepressant duration of use	Antidepressant dose	Psychedelic type	Psychedelic dose	Prior Non-SSRI Psychedelic Experience
"Spider" Report ID: 85489	-	-	-	L	1 tab (Moderate)	No
"420time" Report ID: 70560	Citalopram (Celexa)	-	40mg (Maximum)	L	3 tabs (Very High)	No
"listless" Report ID: 67759	Citalopram (Celexa)	6 months	40mg (Maximum)	P	3.5g (High)	Yes
"Sparky" Report ID: 24962	Citalopram (Celexa)	6 weeks	-	P	10g (Extreme)	Yes
"Lady" Report ID: 109504	Escitalopram (Lexapro)	6 months	15mg (Moderate)	L	1 tab (Moderate)	Yes
"CT" Report ID: 52757	Escitalopram (Lexapro)	-	-	L	-	No
"followthruthefloor" Report ID: 115228	Escitalopram (Lexapro)	-	30mg (Exceeds maximum recommended dose)	P	5g (Very High)	Yes
"Essere" Report ID: 98630	Escitalopram (Lexapro)	-	10mg (Moderate)	P	1.75g (Moderate)	Yes
"The Ovoid Kid" Report ID: 115139	Escitalopram (Lexapro)	1 year	10mg (Moderate)	P, L	12g (Very High)	Yes
"waner" Report ID: 13206	Fluoxetine (Prozac)	-	-	L	3 tabs (Very High)	Yes

"ren" Report ID: 69000	Fluoxetine (Prozac)	-	-	L	15 tabs (Very High)	Yes
"Belle" Report ID: 67323	Fluoxetine (Prozac)	1 year	20mg (Moderate)	L	1 tab (Moderate)	No
"dly2588" Report ID: 64072	Fluoxetine (Prozac)	1 year	20mg (Moderate)	L	1 tab (Moderate)	No
"LinguaSiderea" Report ID: 50678	Fluoxetine (Prozac)	10 years	20mg (Moderate)	L	2 tabs (High)	No
"Craziness" Report ID: 41382	Fluoxetine (Prozac)	-	-	P	-	Yes
"Solipsist" Report ID: 94581	Fluoxetine (Prozac)	-	20mg (Moderate)	P	1.75g (Moderate)	Yes
"urchin" Report ID: 66856	Fluoxetine (Prozac)	6 weeks	20mg (Moderate)	P	-	Yes
"Clay D" Report ID: 103251	Fluoxetine (Prozac)	-	40mg (Maximum)	P	3.5g (High)	-
"JoeShmo" Report ID: 15247	Fluoxetine (Prozac)	Single use (historical prescription)	20mg (Moderate)	P	2g (Moderate)	Yes
"Soquel" Report ID: 93219	Fluoxetine (Prozac)	9 years	40mg (Maximum)	P	3g (High)	No
"Capgras" Report ID: 115347	Fluoxetine (Prozac)	6 months	20mg (Moderate)	P	2g (Moderate)	No
"Andrew C" Report ID: 74557	Fluoxetine (Prozac)	-	10mg (Low)	P	-	Yes
"Bob the Cob" Report ID: 57521	Fluoxetine (Prozac)	2 years	10mg (Low)	P	-	-
"anonymous" Report ID: 107347	Mirtazapine (Noumed)	2 weeks	15mg (Moderate)	L	4 tabs (Very High)	Yes
"Xenon" Report ID: 107458	Paroxetine (Paxil)	-	60mg (Maximum)	L	160µg (High)	Yes

"Tryptamine Fiend" Report ID: 79225	Paroxetine (Paxil)	2 years	20mg (Moderate)	P, L	-, 2 tabs (High)	Yes
"ZoloftShroomer" Report ID: 70865	Sertraline (Zoloft)	1 year	100mg (Moderate)	P, L	1.75g (Moderate), 3 tabs (Very High)	No
"Raizan" Report ID: 111001	Sertraline (Zoloft)	1 month	50-100mg (Moderate)	L	3 tabs (Very High)	-
"kwmi" Report ID: 48587	Sertraline (Zoloft)	1 month	-	P, L	-, 2 tabs (High)	Yes
"Mad Dog" Report ID: 8134	Sertraline (Zoloft)	Single use	400mg (Exceeds maximum recommended dose)	L	2 tabs (High)	-
"Ease" Report ID: 53045	Sertraline (Zoloft)	1 year	25mg (Low)	L	1 tab (Moderate)	No
"Ashton" Report ID: 113828	Sertraline (Zoloft)	-	50mg (Moderate)	P	7g (Very High)	Yes
"N" Report ID: 95018	Sertraline (Zoloft)	Single use	100mg (Moderate)	P	2g (Moderate)	Yes
"desu desu desu" Report ID: 99310	Sertraline (Zoloft)	1 week	25mg (Low)	P	1.8g (Moderate)	Yes
"maizeto" Report ID: 69871	Sertraline (Zoloft)	1 year	150mg (High)	P	3.5g (High)	Yes
"K" Report ID: 8334	Various	2 years	Maximum	P, L	-,-	Yes
"F" Report ID: 35488	Venlafaxine (Effexor)	1 year	112.5mg (Moderate)	P, L	-	Yes
"Matthew 'Holden' Ulm" Report ID: 42405	Venlafaxine (Effexor)	1 year	225mg (Maximum)	L	-	Yes
"Afflatus" Report ID: 9291	Venlafaxine (Effexor)	1 year	275mg (Maximum)	P	3.5g (High)	Yes

"Bran Man" Report ID: 72821	Venlafaxine (Effexor)	2.5 years	-	P	2g-3.5g (Moderate-High)	Yes
"Ligeti" Report ID: 42705	Venlafaxine (Effexor)	1.5 years	150mg (Moderate)	P	1g (Low)	Yes

Note. Dash (-) indicates data not reported. For LSD, low dose = <25 μg (<.5 tab); moderate = 50 μg (1 tab); high = 100 μg (1.5-2 tabs); very high ≥200 μg (≥2 tabs). For psilocybin mushrooms, low dose = <1.5g dried (~<15mg psilocin); moderate = 2.5g (~25mg psilocin); high = 3.5g (35mg psilocin); very high >5g (>50mg psilocin). Dose classifications account for variable psilocybin content across *Psilocybe* species, which typically ranges from 0.5-2% by dry weight. Approximations assume average psilocybin content of 1% per gram dried mushroom, though actual potency may vary significantly. P = psilocybin; L = LSD.

Appendix B

Summary of Main Themes and Subthemes, Alongside Exemplar Quotes

Themes and Subthemes	Description	Exemplars
Altered Acute Effects		
Diminished Hallucinogenic Effects	Reports of diminished or absent visual effects	"I never visibly hallucinated during this trip … Even though I was not hallucinating, the pictures in my mind's eye were distorted." ("Ease" Report ID: 53045) "… the visual patterns that you get seemed to lose their edges. A kind of rounding to everything … The visuals seemed to subside into much more handle-able patterns…" ("K" Report ID: 8334)
Mind-Body Dissociation	Experience of physical effects with reduced intensity of cognitive/mental alterations	"I felt like I was tripping physically for a few hours and couldn't sleep, but my state of mind was absolutely sober." ("F" Report ID: 35488) "The body feeling felt the same as it used to but the hallucinations were diminished as well as the euphoria." ("Tryptamine Fiend" Report ID: 79225)
Temporal Disruption	Alterations in onset timing and duration of psychedelic effects	"A dose that would have resulted in a 3-4 hour experience 3 years ago at that time lasted about 1-1.5 hours." ("The Ovoid Kid" Report ID: 115139)

		"... it lasted about 18-20 hours, when it was only supposed to last from 6-12 hours. I was shocked." (“Belle” Report ID: 67323)
Individual Response Variability	Individual differences in effect reduction, including cases of enhanced effects or full hallucinogenic experiences	"After I began the Paxil I found that the psychedelics lost their magic for me." (“Tryptamine Fiend” Report ID: 79225)
		"Overall a great trip. The one I have been looking for." (“Soquel” Report ID: 93219)
		"… much more intense (especially the body high) than any experience I had without the SSRI." (“kwmi” Report ID: 48587)
Experiential Quality Changes		
Mystical Experiences	Reports of altered consciousness states involving (a) transcendence of space/time, (b) ineffability, (c) euphoric mood, or (d) mystical experiences	"Memories, the past, the future and dreams all became one living experience." (“Essere” Report ID: 98630)
		"Then I noticed that objects took on a new vivacity, a sense that I was perceiving their 'true' from. All objects ... seemed to be communicating their essence in a sort of emotional signal..." (“LinguaSiderea” Report ID: 50678)
		"I experienced a powerful “coming home” type experience ... I was sobbing with joy. I had never before experienced the “coming home” feeling on mushrooms before, nor had I ever cried. The experience was powerfully emotional and cathartic." (“N” Report ID: 95018)

Changed Emotional Qualities	Modifications in emotional processing and attunement	"Flowers opened to the sky in sheer joy. I stress that I was not experiencing hallucinations or even visual illusions … What changed was the emotional valence attached to the environment." (“LinguaSiderea” Report ID: 50678) "I was certainly more in tune with emotions (both mine and hers) than usual." (“Solipsist” Report ID: 94581)
Safety-Seeking Behaviours		
Information Gathering	Efforts to research and understand interaction risks	"I resigned myself to a long trawl through the internet. I hardly found anything on the subject, a few dubious accounts but nothing much." (“Sparky” Report ID: 24962) "I had read a trip report where someone had taken 25mg and 1 tab and had been fine, and my first trip had been fine so I assumed it was safe." (“Raizan” Report ID: 111001)
Safety Planning and Risk Management	Development and implementation of harm reduction strategies, including set/setting considerations	"I had also secreted a note in my pocket explaining the situation to the paramedics. If you're gonna take risks make sure you cover yourself..." (“Sparky” Report ID: 111001) "I felt very safe to do this and I conversed with my boyfriend about it beforehand to make sure we were on the same page and he could keep an eye out." (“Lady” Report ID: 109504) "We decided to stay in a nice safe place (i.e. our home) with no one we felt unfamiliar or insecure with around to limit the chances of a bad trip." (“Spider” Report ID: 85489)

Dose-Response Adaptations		
Intentional Dose Compensation	Planned increases in initial psychedelic dosage to overcome perceived SSRI blunting	"A few days later...I resolved to take the remaining 600ml of tea (60g fresh weight) all at once. Surely, I thought, visual would come from such a dose." ("The Ovoid Kid" Report ID: 115139) "5 is the highest dose of mushrooms I have done, which I took with the expectation that I would be getting a 3 gram dose ... if I accounted for the inhibiting effect of the Lexapro." ("followthruthefloor" Report ID: 115228)
Unplanned Re-dosing	Ad-hoc decisions to take additional doses during the experience	"After the first hit I felt absolutely nothing. About two hours after taking the first hit I took two more…" ("ZoloftShroomer" Report ID: 70865) "Sixty minutes later I still felt nothing, so I took 20g more (200mL)…" ("The Ovoid Kid" Report ID: 115139)
Antidepressant Tapering	Strategic reduction or cessation of serotonergic antidepressant medication before psychedelic use	"10mg is the amount I had been taking each morning, but not since Thursday, and this trip, happened on a Saturday night." ("Andrew C" Report ID: 74557) "I decided to not take my medicine for five days before I dosed ... I finally had my first REAL trip in almost a year (about the time I began my medication)." ("Matthew 'Holden' Ulm" Report ID: 42405)
Potentiation Strategies	Use of additional substances or environmental factors to enhance psychedelic effects	"I smoked some high grade marijuana to see if I could catapult myself into the divine. It did do the job for around a half of an hour …" ("Afflatus" Report ID: 9291)

		"Two grams of Syrian rue seeds marginally increased both duration and magnitude of the experience…" ("The Ovoid Kid" Report ID: 115139)
Adverse Events Spectrum		
Serotonin Toxicity Risk	Occurrence and variation of potential serotonin toxicity symptoms	"... I began staggering about and trembling rather severely ... After a while I felt very cold, then went inside to take a shower to warm myself up … shivering uncontrollably, and felt gripped by hypothermia." ("Mad Dog" Report ID: 8134) "I was struggling to breathe and my mouth was so dry, I was sweating so much and I never had felt so thirsty in my life. I was struggling to swallow and breathe and I couldn't stop crying." ("Raizan" Report ID: 111001)
Psychological Crises	Instances of severe psychological distress, distinguished from typical challenging experiences	"...trapped in your mind with every sight and sound and image and thought and emotion SCREAMING in your mind..." ("JoeShmo" Report ID: 15247) "All I knew was that I was in big fucking trouble. They, with a capital T, were after me and if they caught me my life was effectively over. This crushing feeling of what-have-I-done dominated my consciousness." ("anonymous" Report ID: 107347)
Making Sense Through Comparison		
Social Comparisons	Evaluating effects relative to non-SSRI using peers	"My friends all ate the same amount and were in a completely different state of mind then I was. They were all having a complete trip but nothing happened to me." ("dly2588" Report ID: 64072)

		"I asked B if he felt anything yet. He replied that he “had been tripping this whole time.” Which surprised me, as I had yet to feel much of anything." (“followthruthefloor” Report ID: 115228)
Cross-Substance Comparisons	Contrasting effects between different psychedelic substances	"This same scenario played out every time I ate mushrooms while taking Lexapro. LSD, on the other hand, worked just like it always had before Lexapro. It actually seemed marginally stronger..." (“The Ovoid Kid” Report ID: 115139) "Since starting on Effexor, I have found that the effects of shrooms, and especially acid, are greatly diminished..." (“F” Report ID: 35488)
Expectation versus Reality	Disparities between anticipated and actual effects	"So when he said he tripped balls, I fully believed him and anticipated that I would do the same … I was not under the impression that the effects of tryptamines would become diminished." (“maizeto” Report ID: 69871) "I had been warned that the psilocybin might not be as effective ... Overall, I really enjoyed my acid trip..." (“listless” Report ID: 67759)